Re: HHS Health Sector AI RFI
Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care
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I. Introduction and Commenter Background

I submit this comment to support HHS in identifying concrete actions to accelerate **safe, effective, and equitable** adoption of AI as part of clinical care—particularly in **post-acute and long-term care (PALTC)** settings where readiness barriers are structural and persistent.

My perspective reflects extensive operational experience in PALTC settings, including exposure to interdisciplinary workflows, documentation systems, incident reporting practices, and regulatory compliance pressures across multiple facilities and contexts. My systems-level governance perspective is also informed by my scholarly work, **Human-Led Embodiment & Co-Regulatory Augmentation (H-LECA)**, which emphasizes continuity, risk containment, and human-led control in longitudinal human–AI interaction.

The RFI appropriately centers three federal levers—**regulation, reimbursement, and research & development**—to reduce uncertainty, foster trust, align incentives, and accelerate adoption of AI in clinical care.

II. General Response to the RFI (Cross-Cutting Themes)

A. Readiness is uneven across sectors; PALTC faces distinct foundational barriers

Health care AI adoption is not limited by model capability alone. In PALTC, binding constraints are frequently **infrastructure**, **workforce bandwidth**, **data integrity**, and **organizational instability**—conditions repeatedly identified in long-term care implementation research as barriers to introducing AI-enabled solutions and other digital technologies at scale (Groeneveld et al., 2025; Hassan et al., 2024; Neves et al., 2024; Wong et al., 2024).

B. "Clinical AI" adoption will fail if the first wave is not operationally safe

For PALTC, high-risk “clinical” AI should not be the entry point. A staged acceleration strategy is more likely to be safe and scalable: prioritize **non-clinical workflow AI** (e.g., scheduling, supply chain, documentation quality controls, administrative burden reduction), then progress to higher-risk use only after readiness benchmarks are met. This aligns with evidence that success depends on sociotechnical conditions, workflow integration, and sustained operational capacity rather than tool availability alone (Hassan et al., 2024; Salwei et al., 2021).

C. Continuity, change-management, and post-deployment monitoring should be treated as safety requirements

Predictability, public trust, and risk-proportionate oversight require attention to what happens after deployment: workflow drift, updates, outages, and data shifts can degrade performance and worsen inequities if monitoring is weak (Davis et al., 2025; Subasri et al., 2025). A continuity-aware approach—incorporating detection of destabilizing changes and structured repair pathways—aligns with lifecycle governance expectations for clinical AI (National Institute of Standards and Technology [NIST], 2023).

III. Responses to Specific Questions

1. Biggest barriers to private sector innovation in AI for health care and its adoption/use in clinical care

(a) Infrastructure barriers (PALTC)

- **Unreliable connectivity and unmapped Wi-Fi coverage** in resident care areas.
- Legacy or limited **call-light systems** with failures and limited auditability.
- Environmental and operational conditions that prevent “always available” digital workflows.

Long-term care implementation studies report that infrastructure readiness and organizational capacity strongly shape whether AI-enabled tools can be implemented safely and sustained over time (Groeneveld et al., 2025; Neves et al., 2024; Wong et al., 2024).

(b) Data integrity barriers

- Documentation structures can produce **ambiguous or inconsistent entries** that undermine data reliability for AI training and inference.
- Under high workload, documentation completeness drops, degrading model inputs and increasing downstream risk.

These concerns align with broader evidence that adoption depends on trustworthy data pipelines and workflow-aligned data capture, and that failures frequently arise from sociotechnical breakdowns rather than model architecture alone (Hassan et al., 2024; Salwei et al., 2021).

(c) Workforce and training barriers

- Chronic staffing shortages and turnover reduce capacity for onboarding, calibration, monitoring, and governance.
- Training constraints reduce digital literacy and safe adoption readiness.

Long-term care adoption literature consistently identifies workforce bandwidth, training supports, and staff acceptance as central determinants of success (Hassan et al., 2024; Wong et al., 2024).

(d) Financial and corporate barriers

- Underinvestment in infrastructure and frequent ownership/management turnover disrupt long-horizon modernization planning.

These constraints are consistent with the broader operating environment in skilled nursing facilities and the documented prevalence of ownership changes (Assistant Secretary for Planning and Evaluation, 2024).

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2. What regulatory, payment policy, or programmatic design changes should HHS prioritize, and why? Include CFR citations where applicable.

A. Regulation — baseline digital safety readiness for PALTC

HHS should clarify expectations that facilities meet **minimum digital reliability and governance readiness** before implementing AI-dependent workflows in resident care contexts. One pathway is to leverage existing operational and quality improvement obligations under **42 CFR Part 483**, including:

- **42 CFR § 483.70 (Administration)**
- **42 CFR § 483.75 (Quality Assurance and Performance Improvement; QAPI)**

These sections establish administration and QAPI responsibilities; HHS can provide interpretive guidance and practical readiness benchmarks that support safe, staged adoption and reduce avoidable failures.

B. Regulation — incident reporting and governance alignment

Fragmented definitions and pathways spanning state and federal expectations can increase avoidable harm and legal confusion. HHS can reduce risk by issuing harmonized guidance and model workflows supporting consistent classification and escalation consistent with lifecycle risk management principles (NIST, 2023).

C. Reimbursement — tie incentives to demonstrated readiness and burden reduction

CMS payment and program levers can accelerate adoption by funding readiness first and rewarding tools that reduce administrative burden and improve safety. In PALTC, incentives should prioritize:

- infrastructure modernization and digital reliability
- documentation quality and workflow integrity
- staged implementation linked to measurable readiness thresholds

This approach is consistent with evidence that workflow integration and organizational capacity shape whether technologies reduce burden or add risk (Hassan et al., 2024; Salwei et al., 2021).

D. Programmatic changes — workforce AI literacy and implementation supports

HHS can accelerate adoption by supporting:

- AI literacy programs tailored to PALTC roles (CNA, nursing, administration)
- implementation toolkits (workflow mapping, minimum training, governance templates)
- demonstration projects with explicit safety metrics and transparency requirements

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3. For non-medical devices, what novel legal and implementation issues exist and what role should HHS play?

Key issues include:

- **Liability and accountability ambiguity** among developers, facilities, clinicians, and corporate owners for workflow-support AI outputs.
- **Data provenance risk**: flawed documentation inputs can generate flawed outputs, creating legal exposure and patient harm.
- **Privacy/security operationalization failures**: breakdowns often occur at the workflow layer (access controls, device handling, vendor management), not only the algorithm layer.

HHS's role can include model procurement language, governance templates, and minimum evaluation/monitoring expectations consistent with recognized risk management frameworks for AI systems (NIST, 2023; NIST, 2024).

4. For non-medical devices, promising AI evaluation methods (pre/post), metrics, robustness testing, and how HHS should support

Recommended evaluation stack (pre-deployment through post-deployment)

- human-centered workflow evaluation (usability + burden impact)
- data quality validation (“fitness for use”)
- robustness testing (including outage/fallback behavior)
- bias and error monitoring appropriate to the use case
- post-deployment drift monitoring and incident reporting pathways

These elements align with established reporting and evaluation guidance for clinical AI interventions and trials (Liu et al., 2020; Rivera et al., 2020; Vasey et al., 2022).

Continuity and change-management as safety properties (H-LECA-aligned)

Where AI tools become embedded longitudinally, evaluation should include:

- **continuity expectations** (predictable behavior within defined bounds)
- **rupture handling** (updates/outages/policy shifts communicated clearly; repair pathways exist)
- **auditability and user override** as core criteria for safety and trust

These aims are consistent with emerging evidence on harmful data shifts and fairness drift and the need for proactive post-deployment monitoring (Davis et al., 2025; Subasri et al., 2025).

HHS support mechanisms

HHS can accelerate best practice through:

- cooperative agreements and demonstration pilots
- grants for infrastructure readiness and evaluation capacity
- prize competitions for PALTC-specific safety monitoring tools
- public-private partnerships and applied implementation science

5. How can HHS support private sector accreditation, certification, testing, and credentialing?

HHS can catalyze:

- sector-specific accreditation standards for PALTC readiness (infrastructure + governance)
- certification expectations for workflow AI safety (audit logs, override, downtime behavior, error reporting)
- role-based credentialing for facility staff responsible for AI supervision and monitoring

This approach aligns with recognized needs for consistent governance and lifecycle oversight for AI systems integrated into care delivery (NIST, 2023; Vasey et al., 2022).

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6. Where have AI tools met/exceeded expectations vs fallen short? What novel tools have greatest potential?

From an operational adoption perspective:

- AI has performed better in environments with robust infrastructure and consistent data governance.
- In infrastructure-limited and data-inconsistent settings, tools tied to unreliable inputs or connectivity often fall short or create unacceptable risk.

Implementation literature reinforces that outcomes depend on sociotechnical conditions and workflow fit (Groeneveld et al., 2025; Hassan et al., 2024; Neves et al., 2024; Wong et al., 2024).

Highest near-term potential for PALTC:

- administrative/workflow automation (scheduling, supplies, documentation quality)
- call-light response analytics and alert auditing
- staffing prediction and workload balancing
- carefully validated monitoring tools with explicit privacy constraints

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7. Which roles/decision makers influence adoption, and what are the administrative hurdles?

Influential roles commonly include:

- corporate leadership/ownership
- administrators and Directors of Nursing
- compliance/QAPI leadership and MDS coordination

- EHR/IT vendors and integrators

Primary hurdles include turnover, competing compliance demands, and unclear accountability for technology failures—barriers consistent with organizational-level findings in long-term care technology adoption research (Hassan et al., 2024; Wong et al., 2024).

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8. Where would enhanced interoperability widen market opportunities and accelerate AI?

Priority needs include:

- stronger PALTC ↔ hospital ↔ lab ↔ pharmacy ↔ payer data exchange
- more standardized PALTC documentation structures
- benchmarking datasets and common definitions for outcomes relevant to PALTC safety and quality

Interoperability standards such as HL7 FHIR and associated implementation guidance are consistently identified as key enablers for scalable exchange and AI-ready data integration (Ayaz et al., 2021; Gazzarata et al., 2024; Tabari et al., 2024).

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9. What challenges do patients and caregivers want addressed, and what concerns do they have?

Patients and caregivers commonly seek improved responsiveness, communication, safety monitoring, and consistency in care delivery, while expressing concerns about privacy, reduced human contact, and mistrust when systems are opaque or inconsistent (Neves et al., 2024).

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10. AI research priorities for HHS; and published findings about impacts and cost/benefit approaches

A. Research priorities

- infrastructure readiness and digital reliability as prerequisites to safe AI
- data quality improvement and documentation integrity interventions
- implementation science for workflow AI (burden reduction first)
- post-deployment monitoring, drift detection, and incident reporting methods

- evaluation approaches for monitoring tools used with older adult populations with explicit consent and privacy safeguards

These priorities align with evidence that sustained clinical AI benefit requires rigorous evaluation and ongoing lifecycle oversight, including proactive detection of data shifts and fairness drift (Davis et al., 2025; Liu et al., 2020; Subasri et al., 2025; Vasey et al., 2022).

B. H-LECA as a governance-informed research agenda (appropriate, limited inclusion)

H-LECA does not claim clinical outcomes; its relevance is governance and safety design:

- continuity and behavior stability as a safety target distinct from "memory"
- rupture risk and repair mechanisms (updates/outages/policy shifts)
- auditability, interpretability, override, and bounded delegation in higher-risk workflows

HHS could support research and demonstrations that treat continuity/rupture safeguards as measurable safety properties in longitudinal deployments, consistent with lifecycle governance and monitoring expectations (NIST, 2023; Subasri et al., 2025).

IV. Conclusion

HHS can accelerate adoption of AI in clinical care by pairing innovation with **prerequisite readiness** and **risk-proportionate governance**. For PALTC, a staged approach is most likely to be safe and scalable:

1. modernize infrastructure and data reliability,
2. reduce burden via operational AI first,
3. require rigorous evaluation and post-deployment monitoring, and
4. expand to higher-risk clinical use cases only after readiness benchmarks are met.

A continuity-aware governance lens, including predictable behavior, rupture handling, auditability, and human-led control, can reduce preventable failures that undermine trust and safety in real deployments (Davis et al., 2025; NIST, 2023; Subasri et al., 2025).

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